Version1 26th November 2020

BCUHB Request for supply of off label use of TOCILIZUMAB for a named ADULT patient admitted to ICU with COVID-19 pneumonia

Patient name:		**Site:**	
Hospital number:		**Ward**	
Named consultant:		**DOB:**	

	Please ensure you can answer ALL QUESTIONS indicating that the patient meets the following criteria:	**Yes**
1	I confirm that the patient is ≥18 years old.	
2	The patient has been admitted to ICU/HDU within the last 24 hours with severe pneumonia.	
3	The patient has COVID-19 infection either confirmed by microbiological testing or by MDT review of patient's clinical and radiological features suggesting COVID-19.	
4	The patient requires organ support. (i.e. high-flow nasal oxygen, CPAP, non-invasive/ invasive mechanical ventilation, receiving vasopressor/ inotrope infusions).	
5	The respiratory support was started in the last 24 hours.	
6	I confirm that the patient has not received a dose of tocilizumab during this hospital admission nor are they on long-term therapy with tocilizumab prior to this hospital admission.	
7	I confirm that the patient doesn't have any known hypersensitivity to tocilizumab.	
8	The patient's baseline AST or ALT is not greater than 5 x Upper Limit of Normal (**Use with caution** if LFTs are 1.5 x ULN)	
9	I confirm that the patient doesn't have a co-existing infection (other than COVID-19) that might be worsened by tocilizumab[1]	
10	The patient's baseline platelet count is > 50 x 10^9/L.	
11	The patient's baseline absolute neutrophil count is > than 2 x 10^9/L.	
12	The patient does not have a pre-existing condition or treatment resulting in ongoing immunosuppression.	
13	The patient is not pregnant.[2]	
14	I confirm that I will monitor the patient's eGFR, LFTs and FBC carefully during treatment as clinically appropriate.	
15	I agree to report adverse events, serious adverse events, and special situation reports to MHRA Coronavirus Yellow Card.	
16	I confirm that I will comply with all data collection requirements that are required as part of the mandatory fields of the ISARIC WHO CCP-UK case report form (CRF) scheme.	

DOSE:

- Dose is 8mg/kg to be administered as an intravenous infusion (see dose banding overleaf).
- The total dose should not exceed 800mg.
- Tocilizumab should be diluted in a 100mL bag of 0.9% sodium chloride, after removing an equivalent volume of saline (total volume 100mL) and given over 1 hour[3].
- **A single dose is to be administered, with the option to repeat a dose in 12-24 hours after the initial dose if there has not been sufficient clinical improvement.**
- **Tocilizumab should not be infused concomitantly in the same IV line with other medications.**

Dose banding

Weight*	**Dose**	**Vials**	**Volume of** concentrate for solution for infusion
<41 kg	8mg/kg, rounded to 20mg	As calculated per patient	
≥41 and ≤ 45 kg	360mg	1x 200mg + 2x 80mg	18mL
≥ 46 and ≤ 55 kg	400mg	2x 200mg	20mL
≥ 56 and ≤ 65 kg	480mg	2x 200mg + 1x 80mg	24mL
≥ 66 and ≤ 80 kg	600mg	3x 200mg	30mL
≥ 81 and ≤ 90 kg	680mg	3x 200mg + 1x 80mg	34mL
≥ 91kg	800mg	4x 200mg	40mL

*Rounded to nearest whole kg

Preparations: 80mg in 4mL, 200mg in 10mL, 400mg in 20mL concentrate for solution for infusion

Name of Applicant			
Contact phone no.		**Bleep number**	
Date			

Please email completed form to Teena.Grenier@wales.nhs.uk for DTG information, send a copy to the pharmacy when requesting a supply, and file a copy within the patient notes.

1. Any active, severe infection other than COVID-19; caution is advised when considering the use of tocilizumab in patients with a history of recurring or chronic infections or with underlying conditions which may predispose patients to infections.
2. The REMAP-CAP trial excluded pregnant women, whereas the RECOVERY trial has included pregnant women. SPC for Tocilizumab states: "Women of childbearing potential must use effective contraception during and up to 3 months after treatment. There are no adequate data from the use of tocilizumab in pregnant women. A study in animals has shown an increased risk of spontaneous abortion/embryo-foetal death at a high dose. The potential risk for humans is unknown. Tocilizumab should not be used during pregnancy unless clearly necessary."
3. The REMAP-CAP study protocol recommended: 10ml/hr for first 10mins then 130ml/hr for the remaining 45mins followed by a 20ml sodium chloride 0.9% flush.
4. SPC link: https://www.medicines.org.uk/emc/product/6673/smpc
5. Interim Position Statement: Tocilizumab for patients admitted to ICU with COVID-19 pneumonia (adults). 25th November 2020. https://www.cas.mhra.gov.uk/ViewandAcknowledgment/ViewAlert.aspx?AlertID=103116